
PUBLIC PROGRAMME BRIEF

NWPharmaTech CHR-P initiative

Version **2026-09-26** · Informational overview for visitors, collaborators and press. Not an offering document. Not medical advice.

Version 2026-09-26 · informational only

1. Mission

Intervene earlier in the psychosis pathway — supporting rigorous research aimed at people at clinical high risk — while keeping pharmaceutical-sponsor accountability and clear evidence standards.

2. The need

CHR-P identifies elevated transition risk; it is not a schizophrenia diagnosis and does not mean transition is inevitable. There is still no broadly approved pharmacotherapy labelled to prevent psychosis in CHR-P across major markets.

An updated meta-analysis estimated that about one in four people identified as CHR-P (pooled estimate, 25%) developed psychosis within three years; most did not (Salazar de Pablo et al., 2021). UK NICE guidance does not recommend antipsychotics to prevent psychosis in this group.

3. Investigational programme

NWPT-SM32300 is NWPharmaTech's investigational 300 mg CBD micellar-emulsion softgel for people experiencing symptoms associated with a clinical high risk of psychosis.

A Phase 1 study in 14 healthy volunteers (NCT07186283; ISRCTN25163383) was completed on 10 December 2025. Its results have not yet been published.

The programme is led by Professor Scott Woods as CHR-P programme lead in an individual scientific capacity. See [Team](#) for affiliation notes (Yale University does not sponsor or endorse this website).

The planned Phase 2B study is designed to evaluate **dose response, symptoms, safety and tolerability**, and inform the next stage of development. The proposed design includes approximately **328** participants receiving placebo or one of three daily doses over **12 weeks**, with follow-up at **week 16**.

Effectiveness in this population has **not** been established. Prevention or delay of transition to psychosis is a longer-term question that must be separately powered if claimed as primary. Primary outcome details are being finalised.

4. Evidence status (summary)

- Field CHR-P literature: elevated, heterogeneous risk — cited on the [evidence library](#).
- Other CBD products: hypothesis-generating only — not NWPT-SM32300 efficacy.
- Programme-specific efficacy / prevention: **not established**.
- Numerical exposure and safety figures: not published here until formal study reports are available.

5. Research roadmap

Qualitative horizons only (no invented progress percentages). See the [programme roadmap](#) for communication milestones versus clinical development milestones.

- **Communications (completed where true):** public science resources; programme synopsis / Phase 2B overview; investigational formulation described.
- **Clinical (Phase 2B):** study preparation in progress; applicable approvals not confirmed in public materials; site activation, recruitment, follow-up and analysis planned.

Research roadmap (illustrative)

Not a funding commitment · dates only when accountable

COMPLETED / IN HAND

Formulation concept (SM32300)
Public science resource pages

NEXT PROPOSED

Locked protocol synopsis
CHR-P symptom / functioning PoC

LONGER-TERM

Transition / prevention objectives
if separately powered & financed

Conceptual illustration — research roadmap stages. Website milestones are not clinical progress.

6. Governance

NWPharmaTech remains pharmaceutical sponsor. Investigators, ethics committees and regulators retain independent roles. Community or financing participants do not vote on protocol, dose, endpoints, safety or regulatory submissions.

7. Financing note

A financing route is under evaluation. The preferred path is platform-integrated programme financing, with a permissioned token at close where feasible; a

conventional register-first close remains the fallback. These are development objectives, not live offering terms. This page is not an offer, and no investment is accepted through this website. See [funding use](#) for labelled planning context.

8. Contact

Press and general enquiries: team@nwpharmatech.com. Do not send clinical or personal health information by email.

Selected field references

1. Salazar de Pablo G, Radua J, Pereira J, et al. Probability of transition to psychosis in individuals at clinical high risk: an updated meta-analysis. JAMA Psychiatry. 2021;78(9):970-978.
2. Fusar-Poli P, et al. Meta-analyses of transition risk in clinical high-risk samples (see evidence library).
3. Yung AR, McGorry PD, et al. Ultra-high-risk / CAARMS criteria literature.
4. Cannon TD, et al. NAPLS enrichment / risk prediction research.
5. McGuire P, et al. Am J Psychiatry 2018 — adjunctive CBD in schizophrenia (other product / population).

Full citations and links: [Evidence library](#). Confidential investor diligence materials are not published on this page.

Further reading: [Public synopsis](#) · [Funding use](#) · [Programme](#) · [Contact](#)